

Moment Physical Therapy Weekly Schedule Review

Executive summary of scheduling integrity, provider utilization, and patient retention risk ahead of go-live.

REVIEW PERIOD

July 20–24, 2026

PREPARED FOR

Operations Leadership

STATUS

Action Required

59

Appointments
Reviewed

3

Clinic Locations

6

Providers

24

Patients

The review of the schedule for **July 20–24, 2026** identified several operational issues that could negatively impact patient experience, provider utilization, and clinic efficiency if left unresolved. The analysis covered **59 appointments across 3 clinic locations, 6 providers, and 24 patients**.

4

CRITICAL CONFLICTS

3DATA INTEGRITY
ISSUES**2×**WORKLOAD
IMBALANCE**7**

RECOMMENDATIONS



Critical Scheduling Conflicts

Four high-priority conflicts require correction before the schedule goes live.

1

DOUBLE-BOOKING

Two instances of **provider double-booking**, where a single provider was assigned to two overlapping appointments.

2

IMPOSSIBLE TRANSITION

One **impossible provider transition** between two clinic locations with no travel time built into the schedule.

3

PTO CONFLICT

One provider scheduled **during approved PTO**, resulting in three appointments that require reassignment.



Data Integrity Issues

Inconsistencies affecting scheduling accuracy and downstream administration.

1

DUPLICATE

Duplicate patient appointments were found with incorrect visit numbering.

2

SEQUENCING

A **treatment plan was linked before completion** of the initial evaluation.

3

SYSTEM RISK

An **internal provider ID mismatch** increases the risk of future administrative errors.



Provider Utilization & Continuity of Care

Fragmented continuity and an uneven distribution of workload across providers.

1

CONTINUITY

Patient care continuity was **fragmented by unnecessary provider changes**, despite provider availability.

2

IDLE TIME

One provider remained **completely idle** on a scheduled workday while another absorbed their patients.

3

WORKLOAD

A significant **workload imbalance** was observed — one therapist carried nearly **twice** the appointment volume of another in the same clinic.

R

Patient Retention Risks

Gaps in follow-up that put continuity of care and revenue at risk.



Several patients were **approaching completion of their treatment plans** or had **missed appointments without any follow-up scheduled**, creating a risk of lost continuity of care and revenue.

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Operational Recommendations

Seven actions to strengthen scheduling controls and protect care continuity.

- 01 Resolve all scheduling conflicts**
before the weekly schedule is published.
- 02 Enforce PTO as a scheduling constraint**
rather than a visual reference only.
- 03 Improve data validation**
to prevent duplicate appointments and inaccurate treatment records.
- 04 Balance provider workloads**
while maintaining continuity of care whenever possible.
- 05 Introduce provider travel buffers**
between clinic locations.
- 06 Standardize coverage labeling**
when patients are temporarily reassigned.
- 07 Implement a recurring patient recall process**
for no-shows and patients nearing plan completion.

OVERALL ASSESSMENT

The schedule is generally well organized but requires several targeted corrections before implementation. The most significant opportunities lie in strengthening scheduling controls, improving provider utilization, protecting continuity of care, and introducing proactive patient follow-up processes. Addressing these issues would improve operational efficiency, enhance the patient experience, reduce scheduling errors, and create a more repeatable weekly scheduling review process.